

BISY2004 Project Management

Assessment 2: Evidence Pack

HealthLink EHR: Scenario A, Regional Hospital Digital Records

Item	Details
Group	Group 5
Group members	Robin Hasan Roky, Dilrajpreet Kaur, Ibn Al Obayed Marin, Md Shaparan Omia, Abrar Khan
Website URL	website link here
Website platform	GitHub Pages (HTML/CSS)
Trello board	trello link here
Submitted files	A2_Group5_ProjectLibre.pod · A2_Group5_ProjectData.xlsx · this PDF
Submission date	28/09/2026

1. Tool-Evidence Index and Contribution

Who created or maintained each major artefact.

Artefact	Tool	File / location	Created by	Maintained by
Work Breakdown Structure	diagrams.net	WBS.drawio / Fig 1.1	Robin Hasan Roky	Robin Hasan Roky
Schedule, critical path, baseline, costs	ProjectLibre	A2_Group5_ProjectLibre.pod / Fig 2.1–2.4	Dilrajpreet Kaur	Dilrajpreet Kaur
Budget and earned value dashboard	Google Sheets	ProjectData.xlsx: Budget, Dashboard / Fig 5.6	Dilrajpreet Kaur	Abrar Khan
Risk register and 5×5 heat map	Google Sheets	ProjectData.xlsx: Risks, HeatMap / Fig 3.1–3.2	Ibn Al Obayed Marin	Ibn Al Obayed Marin
Power-interest grid	diagrams.net	Fig 3.3	Ibn Al Obayed Marin	Ibn Al Obayed Marin
Stakeholder register and communication plan	Google Sheets	ProjectData.xlsx / Fig 3.4–3.5	Ibn Al Obayed Marin	Ibn Al Obayed Marin
Quality metrics and change log	Google Sheets	ProjectData.xlsx / Fig 4.1–4.2	Md Shaparan Omia	Md Shaparan Omia
Team chart and RACI	diagrams.net / Google Sheets	Fig 4.3–4.4	Md Shaparan Omia	Md Shaparan Omia
Kanban board	Trello	Board link / Fig 5.1–5.5	Abrar Khan	All members
Issue log	Google Sheets	ProjectData.xlsx / Fig 5.7	Abrar Khan	Abrar Khan
Closure checklist and benefits plan	Google Sheets	ProjectData.xlsx / Fig 6.1–6.2	Abrar Khan	Abrar Khan
Website	GitHub Pages	URL / Fig 6.3	Robin Hasan Roky	All members

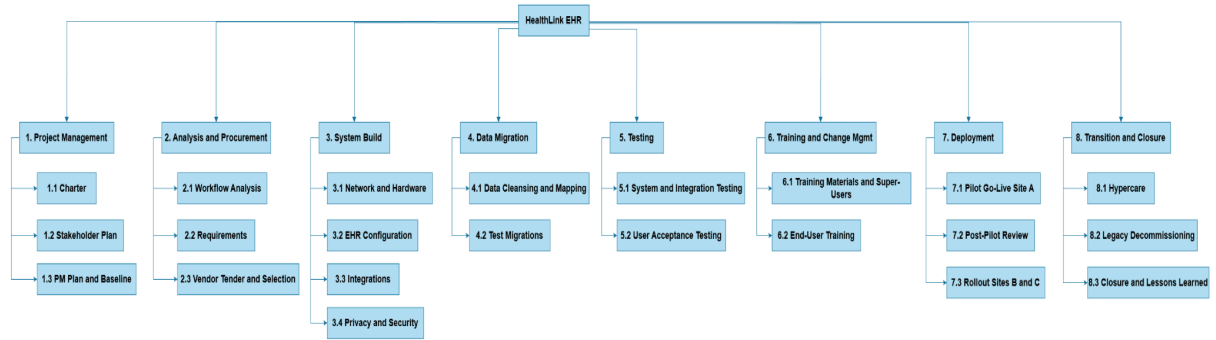
Contribution summary

Member	Main contributions	Website pages / presentation part
Robin Hasan Roky	Project charter, objectives, scope statement and WBS (diagrams.net); set up and published the GitHub Pages website	Overview page; presents project problem, scope and WBS → schedule link
Dilrajpreet Kaur	ProjectLibre schedule: tasks, dependencies, milestones, resources, costs, critical path and baseline; budget and contingency	Schedule & Cost page; presents critical path and what a delay to 3.2 does
Ibn Al Obayed Marin	Risk register and 5×5 heat map; stakeholder register; power-interest grid (diagrams.net); communication plan	Risk & Stakeholders page; presents R01 scoring and stakeholder → risk link
Md Shaparan Omia	Quality metrics, change request log, team structure chart (diagrams.net) and RACI matrix	Quality & Team page; presents CR-02 decision and readiness gate

Member	Main contributions	Website pages / presentation part
Abrar Khan	Trello board setup and updates; earned value dashboard; issue log; closure checklist, benefits plan and lessons learned; compiled evidence pack	Tracking and Recommendation pages; presents variance → corrective action and final recommendation

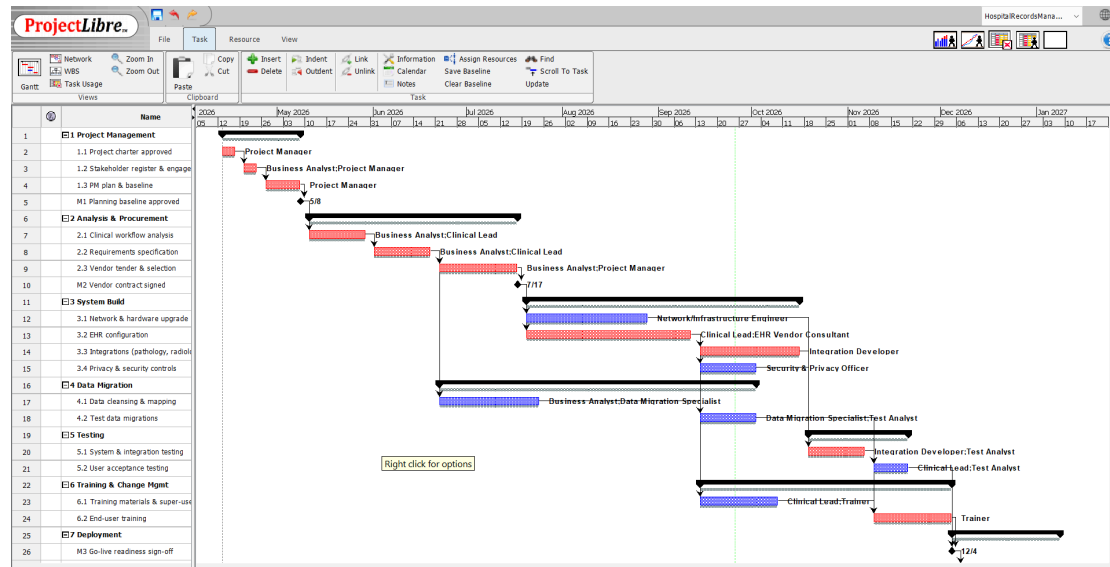
2. Scope

Figure 1.1: Work Breakdown Structure



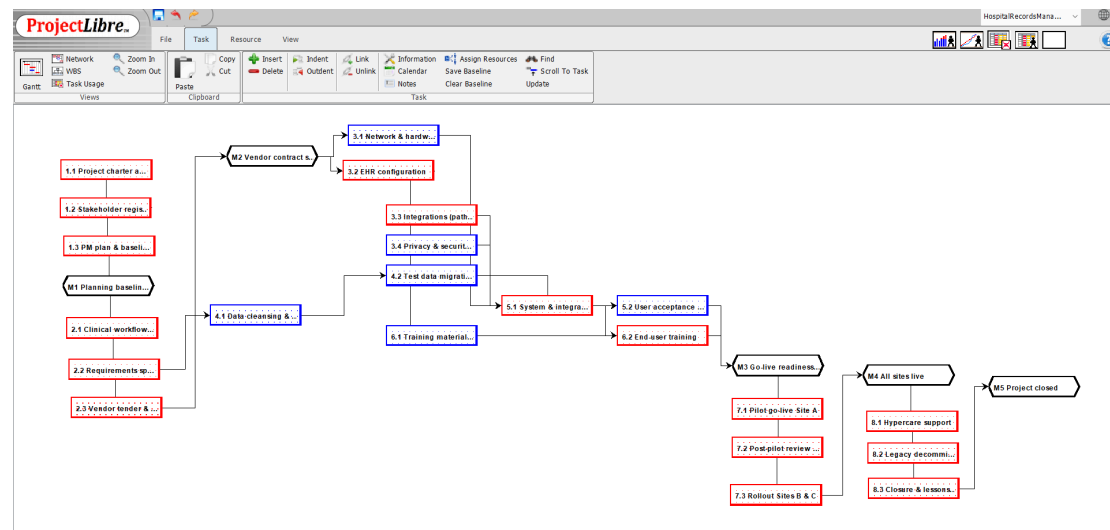
3. Schedule, Cost and Resources

Figure 2.1: Gantt chart with critical path (red) and baseline



Source: ProjectLibre, Gantt view

Figure 2.2: Network diagram



Source: ProjectLibre, Network view

Figure 2.3: Resource sheet with hourly rates

	Name	RBS	Type	E-mail Address	Material Label	Initials	Group	Max. Units	Standard Rate	Overtime Rate	Cost Per Use	Accrue At	Base Calendar
1	Project Manager	Work				P		100%	\$120.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
2	Business Analyst	Work				B		100%	\$95.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
3	Clinical Lead	Work				C		100%	\$110.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
4	EHR Vendor Consultant	Work				E		100%	\$160.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
5	Integration Developer	Work				I		100%	\$130.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
6	Data Migration Specialist	Work				D		100%	\$115.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
7	Network/Infrastructure Engineer	Work				N		100%	\$105.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
8	Security & Privacy Officer	Work				S		100%	\$115.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
9	Test Analyst	Work				T		100%	\$85.00/hour	\$0.00/hour	\$0.00	Prorated	Standard
10	Trainer	Work				T		100%	\$80.00/hour	\$0.00/hour	\$0.00	Prorated	Standard

4. Risk, Stakeholders and Communication

Figure 3.1: Risk register

Risk Register

ID	Risk (cause → event → effect)	Category	Likelihood (1-5)	Impact (1-5)	Score (LxI)	Rating	Owner	Response type	Response action	Linked WBS	Trigger / early warning	Status
R01	Legacy data is incomplete or duplicated, so migrated records are wrong and patient safety is at risk	Data	4	5	20	High	Data Migration Specialist	Mitigate	Cleansing rules, 2 test migrations, clinician sample audit (>99.5% accuracy)	4.1, 4.2	Test migration accuracy below 99.5%	Open
R02	Clinicians resist the new workflow, so adoption is low after go-live	People	4	4	16	High	Clinical Lead	Mitigate	Clinician super-users, involve clinicians in UAT, targeted training	6.1, 6.2	Low training attendance; negative UAT feedback	Open
R03	EHR configuration takes longer than planned, delaying go-live (critical path)	Schedule	3	5	15	High	Project Manager	Mitigate	Fixed-scope sprints, weekly progress review, vendor penalty clauses	3.2	Config sprint misses its target	Triggered – see ISSUE-01
R04	Integration with pathology/radiology/pharmacy fails, so results don't reach clinicians	Technical	3	5	15	High	Integration Developer	Mitigate	Early interface testing, HL7/FHIR standards, vendor support	3.3	Interface test failures	Open
R05	Privacy breach or unauthorised access to patient records	Compliance	2	5	10	Medium	Security & Privacy Officer	Avoid	Role-based access, audit logs, encryption, privacy impact assessment	3.4	Audit log anomalies	Open
R06	Cutover downtime interrupts clinical care	Operational	3	5	15	High	Project Manager	Mitigate	Overnight cutover, paper downtime procedures, rollback plan	7.1, 7.3	Cutover rehearsal overruns	Open
R07	Vendor tender takes longer due to procurement rules	Procurement	3	3	9	Medium	Business Analyst	Mitigate	Early market scan, pre-approved evaluation criteria	2.3	Tender clarifications extend deadline	Closed
R08	Staff can't be released for training because of rostering	People	4	3	12	Medium	Trainer	Mitigate	Short modules, training across shifts, backfill budget	6.2	Booked sessions under 80% full	Open
R09	Hardware or network delivery delayed at smaller sites	Supply	2	3	6	Low	Network/Infrastructure Engineer	Accept	Monitor; 35 days float on 3.1; order early	3.1	Supplier delivery notice slips	Closed
R10	Cost overrun from change requests and scope creep	Cost	3	4	12	Medium	Project Sponsor	Mitigate	Change Control Board, 10% contingency reserve	All	CPI below 0.95	Open
R11	Key vendor consultant leaves mid-project	People	2	4	8	Medium	Project Manager	Transfer	Contract requires vendor replacement within 10 days	3.2, 3.3	Consultant resignation notice	Open
R12	Legacy vendor support ends before decommissioning is complete	Technical	2	4	8	Medium	IT Operations Manager	Mitigate	Negotiate short support extension as fallback	8.2	Go-live slips past Feb 2027	Open

Scoring: Likelihood 1 rare ... 5 almost certain; Impact 1 negligible ... 5 severe (patient safety, go-live, or >10% budget). Rating: High ≥15, Medium 8-14, Low ≤7.

Source: Google Sheets, Risks tab

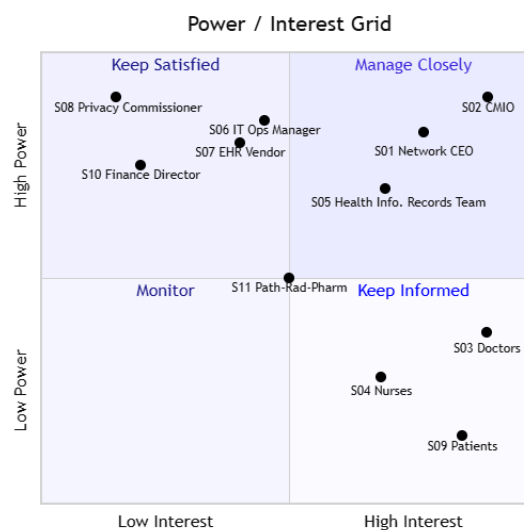
Figure 3.2: 5x5 risk heat map

5x5 Risk Heat Map (number of risks in each cell)

Likelihood ↓ / Impact →	1	2	3	4	5
5	0	0	0	0	0
4	0	0	1	1	1
3	0	0	1	1	3
2	0	0	1	2	1
1	0	0	0	0	0

Source: Google Sheets, HeatMap tab

Figure 3.3: Power-interest grid



Source: diagrams.net export

Figure 3.4: Stakeholder register

Stakeholder Register

ID	Stakeholder	Role / interest in project	Power	Interest	Current engagement	Desired engagement	Strategy (power-interest)	Engagement actions
S01	Network Chief Executive	Project sponsor	High	High	Supportive	Leading	Manage closely	Monthly steering committee; approves budget and go-live
S02	Chief Medical Information Officer	Business owner / clinical sponsor	High	High	Supportive	Leading	Manage closely	Chairs clinical design authority; signs off UAT
S03	Doctors (all sites)	End users	Medium	High	Resistant	Supportive	Keep informed → involve	Clinician champions, UAT involvement, demos
S04	Nurses & Nurse Unit Managers	End users; release staff for training	Medium	High	Neutral	Supportive	Manage closely	Roster-friendly training, super-users on each ward
S05	Health Information / Records team	Data owners	Medium	High	Neutral	Supportive	Keep informed	Data migration working group
S06	IT Operations Manager	Runs infrastructure after handover	High	Medium	Supportive	Supportive	Keep satisfied	Technical working group; BAU handover plan
S07	EHR Vendor	Supplier	High	Medium	Supportive	Supportive	Keep satisfied	Contract meetings, weekly delivery review
S08	Privacy Commissioner / State Health Dept	Regulator	High	Low	Unaware	Neutral	Keep satisfied	Privacy impact assessment shared; compliance reports
S09	Patients & community	Affected by record accuracy and downtime	Low	High	Unaware	Neutral	Keep informed	Hospital newsletter, signage during cutover
S10	Finance Director	Budget control	High	Low	Neutral	Supportive	Keep satisfied	Monthly cost report (CPI, contingency)
S11	Pathology, radiology & pharmacy departments	Integration partners	Medium	Medium	Neutral	Supportive	Keep informed	Interface testing sessions

Source: Google Sheets, Stakeholders tab

Figure 3.5: Communication plan

Communication Plan

Communication	Audience	Content	Method	Frequency	Owner
Steering committee report	Sponsor, CMIO, Finance Director	Status, RAG, SPI/CPI, top risks, decisions needed	Meeting + written report	Monthly	Project Manager
Clinical design authority	CMIO, clinician champions	Configuration decisions, template sign-off	Workshop	Fortnightly	Clinical Lead
Project team stand-up	Project team, vendor	Progress, blockers, Trello board review	Video call	Daily (15 min)	Project Manager
Change Control Board	Sponsor, CMIO, IT Ops Manager	Change requests and impact	Meeting	Fortnightly / as needed	Project Manager
Staff newsletter & intranet	All staff	What's changing, training dates, go-live dates	Email / intranet	Fortnightly	Trainer
Ward huddles	Nurses, doctors	Training reminders, go-live readiness, quick tips	Face to face	Weekly before go-live	Super-users
Vendor delivery review	EHR Vendor	Deliverables, defects, contract milestones	Meeting	Weekly	Project Manager
Regulator compliance update	Privacy Commissioner / State Health Dept	Privacy impact assessment, security controls	Formal letter / report	At milestones M2 and M3	Security & Privacy Officer
Patient notices	Patients & community	Downtime periods, what to expect	Signage, website	2 weeks before each site go-live	Communications Officer

Source: Google Sheets, CommsPlan tab

5. Quality, Change and Team

Figure 4.1: Quality metrics

Quality Requirements and Metrics

ID	Quality requirement	Metric	Target	How / when checked (quality control)	Linked WBS	Owner
Q1	Data migration accuracy	% of sampled records matching source	≥ 99.5%	Sample audit of 500 records per test cycle	4.2, 7.3	Data Migration Specialist
Q2	Critical defects at go-live	Open severity-1 defects	0	Defect log review at readiness sign-off (M3)	5.1, 5.2	Test Analyst
Q3	UAT pass rate	% of UAT scripts passed	≥ 95%	UAT results report	5.2	Clinical Lead
Q4	Staff trained before go-live	% of staff completing training	≥ 95%	Training attendance records	6.2	Trainer
Q5	System availability	Uptime during hypercare	≥ 99.9%	Vendor monitoring report	8.1	IT Operations Manager
Q6	Record retrieval time	Time to open full cross-site record	< 2 minutes	Timed sample, 3 months after go-live	Benefit B1	Clinical Lead

Source: Google Sheets, Quality tab

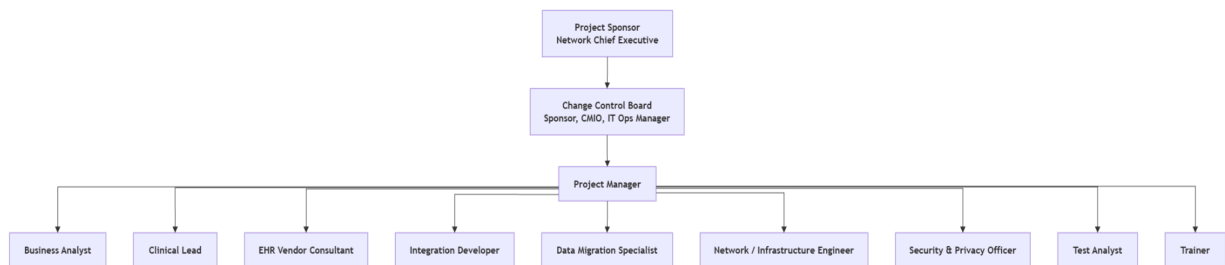
Figure 4.2: Change request log

Change Request Log

CR ID	Date raised	Raised by	Change requested	Impact (scope / time / cost)	Risk impact	Decision	Decided by / date
CR-01	14 Aug 2026	CMIO	Add radiology image viewer to clinician workspace	Scope +; +5 days on 3.3 (non-critical at the time); +\$12,000	Low (uses existing interface)	Approved	CCB 21 Aug 2026
CR-02	18 Sep 2026	Project Manager	Add second vendor consultant to 3.3 Integrations to recover 5 of 10 days lost on 3.2	Schedule -5 days; cost +\$25,600 (20 days x 8 h x \$160) from contingency	Reduces R03/R04 exposure	Approved	CCB 22 Sep 2026
CR-03	21 Sep 2026	Site B Director	Add patient-facing mobile app	Out of scope (exclusion); est. +\$400,000 and +60 days	High	Rejected – refer to future project	CCB 22 Sep 2026

Source: Google Sheets, ChangeLog tab

Figure 4.3: Project team structure



Source: diagrams.net export

Figure 4.4: RACI matrix

RACI Matrix (R = Responsible, A = Accountable, C = Consulted, I = Informed)

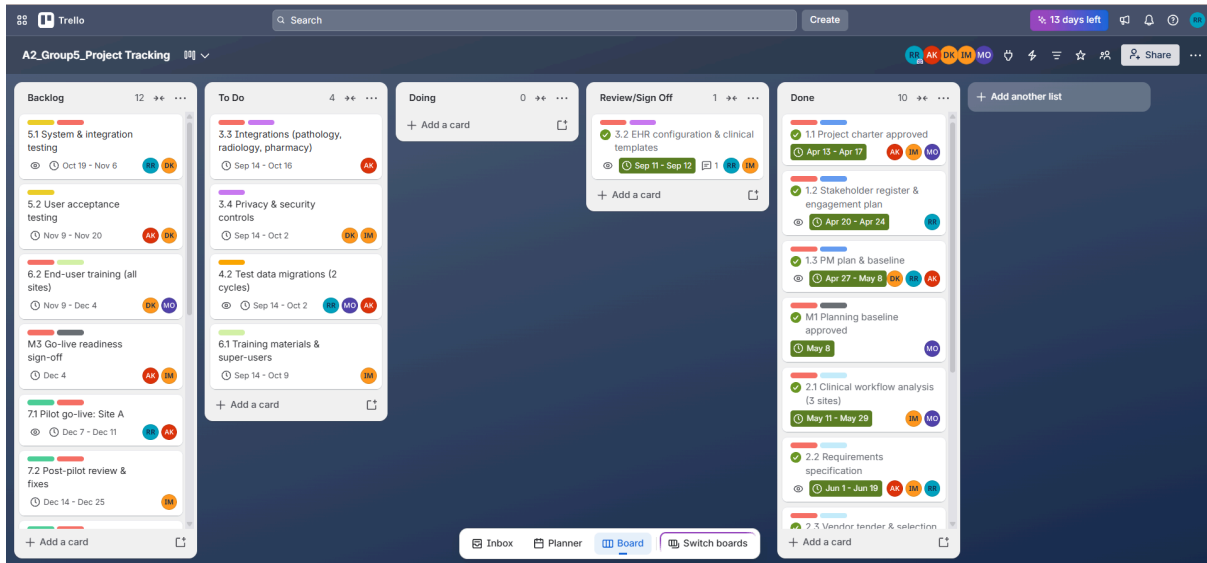
Deliverable	Sponsor	CMIO	Project Manager	Business Analyst	Clinical Lead	EHR Vendor	IT Ops / Infrastructure	Security & Privacy Officer	Trainer
Project charter	A	C	R	C	C	I	I	I	I
Requirements specification	I	A	C	R	R	C	C	C	I
Vendor selection	A	C	R	R	C	I	C	C	I
EHR configuration	I	A	C	C	R	R	I	C	I
Integrations	I	C	A	C	C	R	R	C	I
Data migration	I	C	A	R	C	R	C	C	I
Privacy & security controls	I	C	A	I	I	C	C	R	I
Testing & UAT	I	A	C	C	R	R	C	C	I
Training	I	C	A	I	C	C	I	I	R
Go-live decision	A	R	R	I	C	C	C	C	I
Project closure	A	C	R	C	C	I	C	I	I

Rule check: each deliverable has exactly one A.

Source: Google Sheets, RACI tab

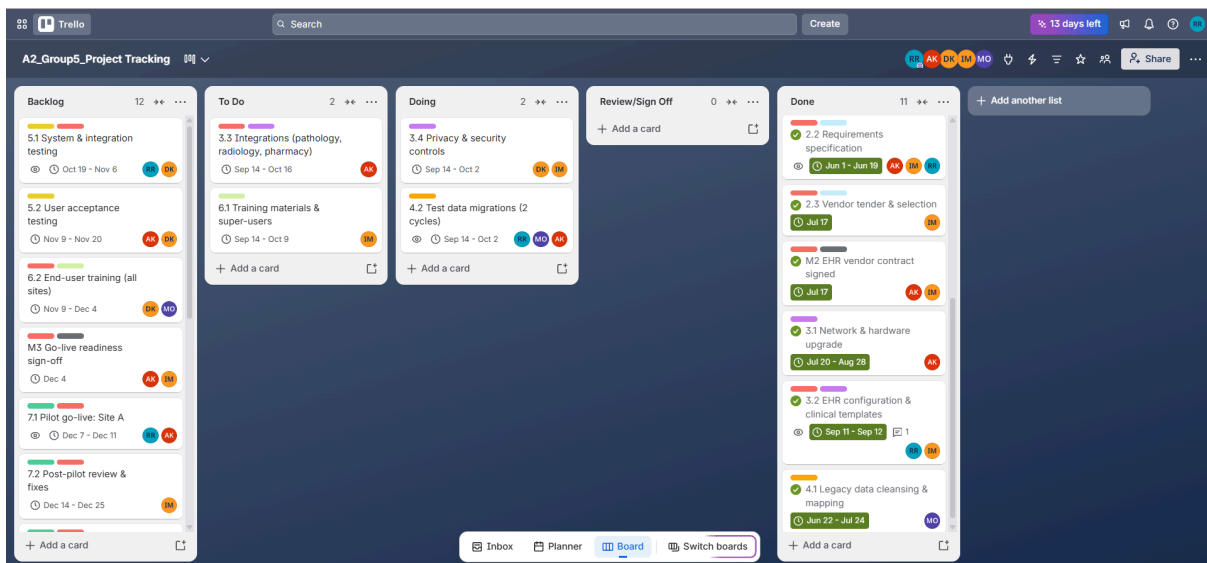
6. Tracking and Dashboard

Figure 5.1: Trello board at status date, 25 Sep 2026 (T4)



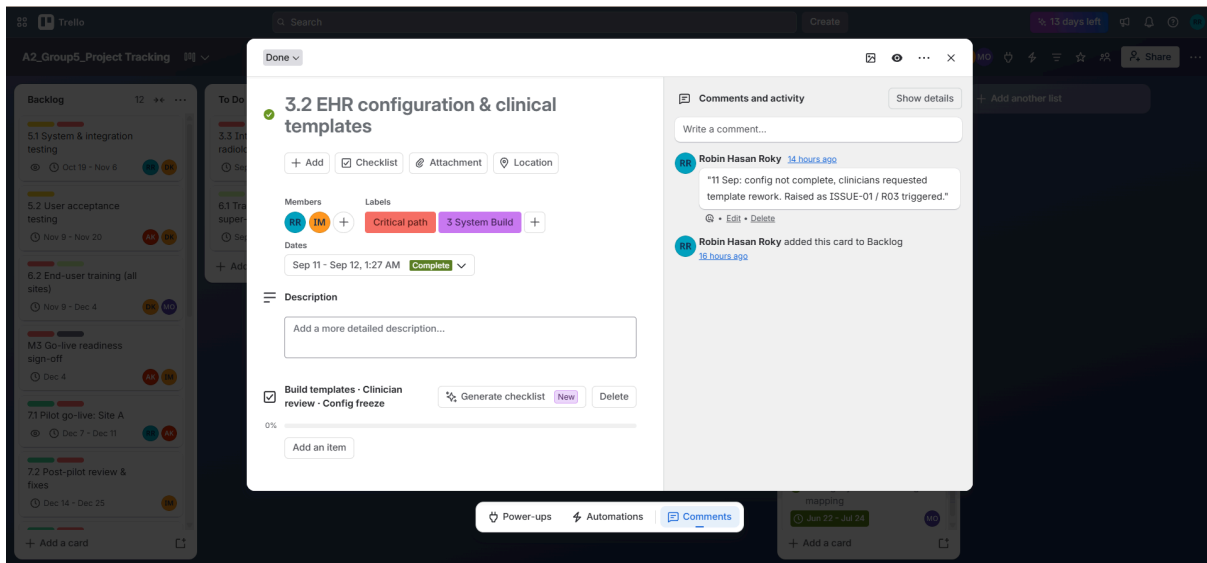
Source: Trello

Figure 5.2: Trello board one week later, 2 Oct 2026 (T6)



Source: Trello

Figure 5.3: Card 3.2 detail: labels, dates, checklist (T2)



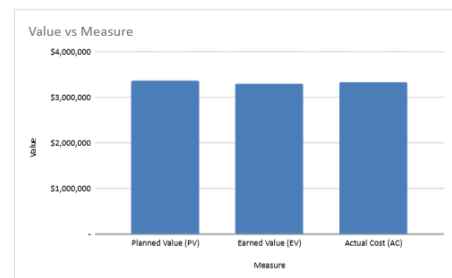
Source: Trello

Figure 5.4: Earned value dashboard with chart

Project Status Dashboard – status date Fri 25 Sep 2026

Measure	Value	What it means
Budget at Completion (BAC)	\$3,550,600	Total planned cost of the project (from ProjectLibre costs)
Planned Value (PV)	\$3,361,001	Value of work that should be done by 25 Sep
Earned Value (EV)	\$3,310,200	Value of work actually done by 25 Sep
Actual Cost (AC)	\$3,331,800	What the completed work actually cost
Schedule Variance SV = EV - PV	(\$50,801)	Negative = behind schedule
Cost Variance CV = EV - AC	(\$21,600)	Negative = over budget
Schedule Performance Index SPI = EV / PV	0.98	Below 1.00 = behind schedule
Cost Performance Index CPI = EV / AC	0.99	Below 1.00 = over budget
Estimate at Completion EAC = BAC / CPI	\$3,573,769	Forecast final cost if current cost performance continues
Variance at Completion VAC = BAC - EAC	(\$23,169)	Forecast over/under run at completion
Critical path slip (working days, from ProjectLibre)	10	3.2 finished 10 days late and is on the critical path
Days recovered by CR-02 (forecast)	5	Second vendor consultant on 3.3 (approved change CR-02)
Forecast go-live slip (working days)	5	Remaining delay to go-live forecast
Schedule RAG	AMBER	Rule: RED if SPI<0.95 or slip>10 days; AMBER if SPI<1 or any slip
Cost RAG	AMBER	Rule: RED if CPI<0.95, AMBER if CPI<1

Interpretation: SPI and CPI look close to 1.00 because the large fixed costs (licences, hardware) were already spent on time. The real problem is the critical path: 3.2 finished 10 days late, which pushes go-live back unless corrected. Look at the slip in days, not just the indices.



Source: Google Sheets, Dashboard tab

Figure 5.5: Issue log

Issue Log

ID	Date raised	Issue	Priority	Impact	Owner	Corrective action	Status
ISSUE-01	11 Sep 2026	3.2 EHR configuration not complete; clinicians requested template rework	High	Critical path slips 10 working days; go-live at risk	Project Manager	CR-02 approved: second vendor consultant on 3.3; start 6.1 in parallel with 3.3	Resolved 25 Sep 2026 (3.2 complete, 10 days late); schedule recovery in progress
ISSUE-02	29 Sep 2026	Pharmacy system vendor late supplying interface specification	Medium	Part of 3.3 blocked; could erode the 5 days recovered	Integration Developer	Escalated to vendor account manager; build pathology and radiology interfaces first	Open – due 6 Oct 2026
ISSUE-03	22 Sep 2026	Site C training room unavailable in November	Low	6.2 training at Site C may slip	Trainer	Book community clinic room; add online modules	Open

Source: Google Sheets, IssueLog tab

7. Closure and Website

Figure 6.1: Closure checklist

Project Closure Checklist

Closure item	Owner	Status
Sponsor acceptance of all deliverables	Sponsor	Planned
All sites live and stable (M4)	Project Manager	Planned
Hypercare exit criteria met	IT Operations Manager	Planned
Open defects handed to BAU support	IT Operations Manager	Planned
Legacy systems decommissioned and data archived	IT Operations Manager	Planned
Vendor contract deliverables confirmed and final payment	Finance Director	Planned
Final cost report and contingency reconciled	Project Manager	Planned
Lessons learned workshop held and recorded	Project Manager	Planned
Benefits owners confirmed and measurement scheduled	Sponsor	Planned
Project documents archived	Project Manager	Planned
Team released and thanked	Project Manager	Planned

Source: Google Sheets, Closure tab

Figure 6.2: Benefits realisation plan

Benefits Realisation Plan

ID	Benefit	Measure	Baseline	Target	When measured	Benefit owner
B1	Faster access to full patient records	Time to retrieve cross-site record	~20 min	< 2 min	3 months after go-live	CMIO
B2	Fewer medication errors from missing information	Medication incidents per 1,000 admissions	Baseline FY2026	~20%	12 months after go-live	Director of Pharmacy
B3	Less duplicate testing	Repeat pathology tests within 48 h	Baseline FY2026	~15%	12 months after go-live	Pathology Director
B4	Lower legacy support costs	Annual support and licence cost of old systems	4 systems	Decommissioned	After 8.2	IT Operations Manager
B5	Better compliance and audit	Records with complete audit trail	Partial	100%	At go-live	Security & Privacy Officer

Source: Google Sheets, Benefits tab

Figure 6.3: Website home page

← → ↺

robinhasanroky.github.io/healthlink-ehr/

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HealthLink EHR: Project Management Site

BISY2004 Assessment 2 · Group XX · Scenario A: Regional Hospital Digital Records

[Overview](#)[Schedule & Cost](#)[Risk & Stakeholders](#)[Quality & Team](#)[Tracking](#)[Recommendation](#)[Evidence](#)

Project Overview, Charter & Scope

Why does the hospital network need this project, and what exactly will it deliver?

3 sites
one integrated EHR

230 days
13 Apr 2026 – 26 Feb 2027

\$3.91M
approved budget

Hybrid
predictive + iterative

The problem

A regional hospital network with three sites runs four separate legacy systems for patient administration, pathology, pharmacy and paper notes. Clinicians can't see a patient's full record across sites, handovers rely on phone and paper, and two systems lose vendor support within 18 months.

Purpose: HealthLink EHR replaces these systems with one integrated electronic health record, available securely at all three sites.

Approach: a hybrid life cycle. Procurement, infrastructure, migration and rollout follow a predictive, baselined plan. EHR configuration and testing run in short iterations with clinicians, because clinical templates need repeated feedback. Rollout is phased: pilot at Site A, then Sites B and C.

Source: GitHub Pages (published site)

8. References (APA 7th)

OAIC. (2023). *Australian privacy principles*. Office of the Australian Information Commissioner.

<https://www.oaic.gov.au/privacy/australian-privacy-principles>

Project Management Institute. (2021). *A guide to the project management body of knowledge (PMBOK guide)* (7th ed.). Project Management Institute.

Projectlibre. (2024). *Home | projectlibre*. Www.Projectlibre.Com. <https://www.projectlibre.com/>

Trello. (2025). *Trello*. Trello. <https://trello.com/>